

Product Requirements Document (PRD)

Hospital Bed Allocation System

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Status: Draft

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Target: College DBMS Course Project

1. Executive Summary

1.1 Product Vision

Build a desktop-based Hospital Bed Allocation System that demonstrates core Database Management System (DBMS) concepts through a functional, transactional application. The system manages hospital bed inventory, patient admissions, discharges, waitlists, and generates occupancy reports.

1.2 Target Users

- **Hospital Receptionist / Admin Staff:** Primary users who admit patients, allocate beds, process discharges, and view reports.
- **System Administrator:** Manages ward configurations, bed inventory, and staff records.

1.3 Success Criteria

- All beds can be tracked in real-time (Available, Occupied, Maintenance).
 - Patient admission and discharge are atomic database transactions.
 - The system demonstrates 3NF/BCNF normalization, triggers, stored procedures, views, and constraints.
 - Waitlist is automatically processed when a bed becomes available.
 - Swing GUI provides color-coded visual bed status.
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2. Scope

2.1 In Scope

- Ward and bed inventory management
- Patient registration and admission workflow
- Bed allocation with ward-type preference
- Patient discharge and bed deallocation
- Waitlist management with priority scoring
- Bed maintenance scheduling
- Staff (doctor/nurse) assignment tracking
- Occupancy and turnover reports
- Audit trail for bed status changes

2.2 Out of Scope

- Billing and payment processing
- Electronic Medical Records (EMR) / clinical notes
- Pharmacy inventory management
- Multi-hospital deployment
- Web/mobile interfaces
- Role-based access control beyond basic admin/receptionist
- Real-time IoT sensor integration

3. Functional Requirements

3.1 Ward & Bed Management (FR-BED)

ID	Requirement	Priority
FR-BED-01	System shall store ward details: Ward ID, Ward Type (General, ICU, Emergency, Private), Floor, Capacity, Daily Charge	High
FR-BED-02	System shall store bed details: Bed ID, Ward ID, Bed Number, Equipment Status, Current Status (Available, Occupied, Maintenance)	High
FR-BED-03	System shall allow adding new wards and beds via admin panel	Medium
FR-BED-04	System shall allow marking a bed as “Maintenance” and logging the reason	Medium
FR-BED-05	System shall prevent allocating a bed that is Occupied or under Maintenance	High

3.2 Patient Management (FR-PAT)

ID	Requirement	Priority
FR-PAT-01	System shall store patient details: Patient ID, Name, Age, Blood Group, Contact, Address, Emergency Contact	High
FR-PAT-02	System shall allow registering a new patient before admission	High
FR-PAT-03	System shall maintain patient admission history (multiple admissions over time)	High

Table 2 – continued

ID	Requirement	Priority
FR-PAT-04	System shall support searching patients by ID or Name	Medium

3.3 Admission & Allocation (FR-ADM)

ID	Requirement	Priority
FR-ADM-01	System shall allow admitting a patient by selecting ward type preference	High
FR-ADM-02	System shall automatically find and allocate the first available bed matching the ward type	High
FR-ADM-03	System shall record admission timestamp, expected discharge date, assigned doctor, and bed ID	High
FR-ADM-04	System shall atomically update bed status to “Occupied” upon successful admission	High
FR-ADM-05	System shall handle concurrent allocation requests without double-booking (transaction isolation)	High
FR-ADM-06	If no bed is available, system shall add patient to waitlist with priority score	High
FR-ADM-07	Priority score = (Severity Level $\times$ 100) + Wait Time in hours	Medium

3.4 Discharge & Deallocation (FR-DIS)

ID	Requirement	Priority
FR-DIS-01	System shall allow discharging a patient from an occupied bed	High
FR-DIS-02	System shall record actual discharge timestamp and calculate stay duration	High
FR-DIS-03	System shall atomically free the bed (status $\rightarrow$ Available or Maintenance)	High

Table 4 – continued

ID	Requirement	Priority
FR-DIS-04	Upon discharge, system shall automatically check waitlist and allocate bed to highest-priority waiting patient of matching ward type	High
FR-DIS-05	System shall generate a discharge summary with bed turnover time	Medium

3.5 Waitlist Management (FR-WAI)

ID	Requirement	Priority
FR-WAI-01	System shall display waitlist sorted by priority score (descending)	High
FR-WAI-02	System shall allow manual removal of a patient from waitlist	Medium
FR-WAI-03	System shall show estimated wait time based on average turnover per ward type	Low
FR-WAI-04	When a bed is freed, system shall auto-allocate to the highest-priority waitlisted patient for that ward type	High

3.6 Staff Management (FR-STF)

ID	Requirement	Priority
FR-STF-01	System shall store staff: Staff ID, Name, Role (Doctor/Nurse), Department, Phone	Medium
FR-STF-02	System shall link a doctor to each admission record	Medium
FR-STF-03	System shall display doctor workload (number of current admissions per doctor)	Low

3.7 Reporting & Analytics (FR-RPT)

ID	Requirement	Priority
FR-RPT-01	Dashboard showing total beds, occupied count, available count, maintenance count per ward	High
FR-RPT-02	Current occupancy rate per ward type (percentage)	High
FR-RPT-03	Bed turnover report: average stay duration per ward type	Medium
FR-RPT-04	Waitlist summary: count per ward type, longest waiting patient	Medium
FR-RPT-05	Doctor workload report	Low

3.8 Audit & Logging (FR-AUD)

ID	Requirement	Priority
FR-AUD-01	Every bed status change shall be logged with timestamp, old status, new status, and triggering event	Medium
FR-AUD-02	Admission and discharge events shall be immutable records	High

4. Non-Functional Requirements

4.1 Performance (NFR-PERF)

- Bed allocation query shall execute within 2 seconds for up to 500 beds.
- Dashboard data refresh shall take < 3 seconds.
- Waitlist auto-allocation trigger shall execute within 1 second of discharge.

4.2 Reliability (NFR-REL)

- Database transactions for admission/discharge must be ACID-compliant.
- No data loss on concurrent allocation attempts.

4.3 Usability (NFR-UI)

- Swing GUI shall use color coding: Green (Available), Red (Occupied), Yellow (Maintenance), Gray (Other).
- All forms shall have input validation with clear error messages.
- Table views shall support sorting by clicking column headers.

4.4 Maintainability (NFR-MNT)

- SQL schema, triggers, and procedures shall be stored in separate .sql files.
- Java code shall follow DAO pattern with clear separation of GUI and database layers.

4.5 Portability (NFR-PORT)

- System shall run on any machine with JDK 17+ and MySQL 8.0+ / PostgreSQL 14+.
 - Database connection parameters shall be externalized in a config file.
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5. User Interface Requirements

5.1 Screens

1. **Login Screen:** Username/password (simplified for college demo).
2. **Main Dashboard:**
 - Top bar: Total stats (Total Beds / Occupied / Available / Maintenance).
 - Center: `JTable` of all beds with color-coded rows.
 - Side panel: Quick action buttons (Admit, Discharge, Waitlist, Reports).
3. **Admit Patient Dialog:**
 - Patient search/registration form.
 - Ward type preference dropdown.
 - Doctor assignment dropdown.
 - “Allocate Bed” button with confirmation.
4. **Discharge Dialog:**
 - List of currently occupied beds with patient names.
 - “Discharge” button.
 - Post-discharge: option to mark bed for maintenance.
5. **Waitlist Manager:**
 - `JTable` of waitlisted patients with priority scores.
 - “Auto-Allocate” and “Remove” buttons.
6. **Reports Screen:**
 - Tabbed interface: Occupancy, Turnover, Doctor Workload.
 - Each tab displays a `JTable` populated from database Views.
7. **Admin Panel:**
 - Forms to add/edit wards, beds, and staff.

5.2 GUI Standards

- Use `JTable` with custom `TableCellRenderer` for color coding.
 - Use `JOptionPane` for confirmations and error alerts.
 - Use `JComboBox` for dropdown selections (ward type, doctor, bed status).
 - Use `JDatePicker` or `JFormattedTextField` for date inputs.
 - Window size: 1024×768 minimum, resizable.
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6. Data Requirements

6.1 Data Entities

- **Ward:** ward_id, ward_type, floor, capacity, daily_charge
- **Bed:** bed_id, ward_id, bed_number, equipment_status, current_status
- **Patient:** patient_id, name, age, blood_group, contact, address, emergency_contact

- **Staff:** staff_id, name, role, department, phone
- **Admission:** admission_id, patient_id, bed_id, doctor_id, admission_date, expected_discharge, actual_discharge, status
- **Waitlist:** waitlist_id, patient_id, requested_ward_type, priority_score, request_time, status
- **BedMaintenanceLog:** log_id, bed_id, start_time, end_time, reason, resolved_by

6.2 Data Volumes (Expected)

- Wards: 4–10 records
- Beds: 20–100 records
- Patients: 50–500 records
- Admissions: 100–1000 records (historical + active)
- Staff: 10–50 records
- Waitlist: 0–20 active records at any time

7. Constraints & Assumptions

7.1 Constraints

- Single-user or single-workstation deployment (college demo scope).
- No network authentication (local DB user).
- No encryption required for demo data.
- Swing is the only permitted GUI framework.

7.2 Assumptions

- Hospital operates 24/7; system does not need shift scheduling.
- One patient per bed at any given time.
- Bed equipment status is manually updated by admin.
- Severity level is manually entered by receptionist (1–5 scale).

8. Risks

Risk	Mitigation
Concurrent bed allocation race condition	Use JDBC transactions with proper isolation level (SERIALIZABLE or pessimistic locking)
Database connection failure	Implement connection retry logic and show user-friendly error dialog
Swing GUI becomes unresponsive during DB operations	Run DB queries in <code>SwingWorker</code> background threads
Data inconsistency from manual status updates	Enforce bed status changes only through triggers/procedures

9. Deliverables

1. **Source Code:** Java Swing application with JDBC DAO layer.
 2. **SQL Scripts:** `schema.sql`, `triggers.sql`, `procedures.sql`, `views.sql`, `seed_data.sql`.
 3. **Database Design Document:** ER Diagram, Normalization steps (UNF $\rightarrow$ BCNF), FDs.
 4. **Project Report:** Screenshots, SQL query samples, DBMS concepts explanation.
 5. **Executable JAR:** Runnable demo application.
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10. Glossary

Term	Definition
Ward Type	Category of hospital ward: General, ICU, Emergency, Private
Bed Status	Current state: Available, Occupied, Maintenance
Priority Score	Computed value determining waitlist order
Turnover Time	Duration between discharge and next admission for a bed
DAO	Data Access Object —Java pattern for database operations
ACID	Atomicity, Consistency, Isolation, Durability

End of PRD